



Policy

Sexual Safety in Mental Health

Policy code	EMHS: 312
Original endorsement date	August 2023
Date of current endorsement	July 2025
Version	2.0
Functional subgroup	Mental Health
Summary	This policy provides direction to EMHS mental health services regarding the establishment and maintenance of the sexual safety of people accessing mental health services, support people, staff and others accessing the services.
Policy owner	Executive Director Mental Health
Contact	
Review date	July 2028
NSMHS Standards	
Impact Statement Declaration (ISD)	The Aboriginal community is reflected in this policy to ensure that cultural appropriateness and positive health impacts on Aboriginal people have been considered and incorporated (ISD Record ID4969).
Status	Active
Information Classification	OFFICIAL

1. Background

Sexual safety incidents and violence are crimes that have long term consequences for their victims. While these types of crimes potentially affect all members of the community, research confirms that people experiencing mental health symptoms or impairment are at a considerably higher risk due to a number of factors, including a past or current experience of abuse (including sexual abuse). People with a past history of trauma may also experience aspects of admission and mental health care as traumatic.

This makes sexual safety an essential component for people who use a mental health service – whether the person is receiving treatment in a hospital setting, a rehabilitation or residential setting, or within the community.

A sexually safe environment is culturally safe, and trauma informed. It emphasises recovery and is based on principles of safety, trustworthiness, choice, collaboration and empowerment. Mental health professionals need to be aware of possible past sexual safety incidents and how this may increase a person's vulnerability to further inappropriate sexual activity, exposure or exploitation by others.

This policy aims to:

- Establish expected standards for the sexual safety of people accessing mental health services in all health care settings.
- Clearly outline the responsibilities of mental health services in relation to establishing and maintaining the sexual safety of people accessing mental health services whilst supporting them to adopt practices and behaviours that contribute to their sexual safety.
- Acknowledge the importance in supporting the privacy and dignity of people accessing mental health services.
- Develop a consistent, co-ordinated approach to the promotion of sexual safety and the prevention of and response to sexual safety incidents.
- Provide staff with adequate information to enable them to effectively promote strategies to support sexual safety and to respond appropriately, consistently and sensitively to sexual safety issues, and
- Improve the sexual safety of people accessing mental health services, personal support people, staff and others accessing the services.

Policy exclusion: sexual contact between people accessing mental health services and staff is excluded from this policy. If sexual contact between staff and people accessing mental health services is suspected, staff should inform their line manager and/ or EMHS Integrity;

emhs.integrityðics@health.wa.gov.au.

1.1. Key definitions

Term	Definition
Consensual sexual activity	Sexual activity that occurs after mutual sexual consent has been provided by those involved. It includes any activity of a sexual nature (touching, intercourse, oral sex, kissing) that occurs between people over the age of 16 years after mutual consent has been provided, who have been assessed as having the capacity to consent.
Gender	Gender refers to the socially constructed roles, behaviours, expressions and identities of girls, women, boys, men and gender diverse people. It influences how people perceive themselves and each other, how they act and interact. There is considerable diversity in how individual people and groups understand, experience and express gender. A person's sex and gender may not necessarily be the same. A person's preferred gender may or may not correspond with the sex or gender assigned at birth and some people may identify as neither male nor female (i.e., non-binary).
Gender sensitive practices	Practice that recognises and responds to the differences, inequalities and specific needs across the gender spectrum.
Inappropriate context or setting	This refers to sexual activity between people accessing mental health services, or people accessing mental health services and their visitors, taking place in an environment or under a set of circumstances that is not considered to be suitable. This includes masturbation, within any areas public or private, such as: • An acute inpatient setting • Emergency Department • Public areas • Outpatient/non-acute setting • Or any other area, either public or private, within a health care setting.
Informed consent	A decision made freely and voluntarily by a person with the cognitive capacity to understand the nature, consequences, and risks of a sexual act.
Perpetrator/offender	A person who has breached the sexual safety of another person.
Sexual activity	Activity of a sexual nature with oneself (masturbation) or another (sexual touching, sexual intercourse, oral sex, use of sexually explicit magazines, videos/other recordings, use of sex toys etc).

Sexual assault	A crime involving any unwanted sexual act that occurs without consent, including physical contact of a sexual nature.
Sexually disinhibited behaviour	Poorly controlled behaviour of a sexual nature where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way, such as in inappropriate situations; at the wrong time, in the wrong setting, or with the wrong person.
Sexual harassment	Unwelcome conduct of a sexual nature which makes a person feel offended, humiliated and/or intimidated where that reaction is reasonable in the circumstances. Can involve physical, visual, verbal or non-verbal conduct.
Sexual health	A state of physical, emotional, mental and social well-being related to sexuality, including the absence of disease, dysfunction or infirmity; a positive and respectful approach to sexuality and sexual relationships; the possibility of having pleasurable and safe sexual experiences, free from coercion, discrimination and violence, and respect for the sexual rights of all people.
Sexual safety	When a person feels psychologically, emotionally and physically safe, including feeling safe from behaviour of a sexual nature that is unwanted, or makes the person feel uncomfortable, afraid or unsafe.
Sexual safety incident	The term used to refer to an incident that breaches or compromises the sexual safety of a person accessing mental health services, and which is recognised as either sexual assault or harassment, consensual sexual activity in an inappropriate setting or sexually disinhibited behaviour.
Support person	A carer, close family member, parent, guardian or nominated person of someone accessing mental health services who is involved in supporting them.
Trauma informed care	Treatment directed by: • A thorough understanding of the profound neurological, biological, psychological and social effects of trauma and violence on the individual; and • An appreciation for the high prevalence of traumatic experiences in people accessing mental health services.

1. Responsibilities and Minimum Requirements

2.1. Responsibilities

EMHS Mental health services have a responsibility to:

- Define and promote the appropriate standard of behaviour expected of people accessing mental health services, support people and staff involved with the service.
- Promote the rights and responsibilities of everyone involved in the service in relation to sexual safety, including reporting and disclosing sexual safety incidents.
- Ensure information about sexual safety, and available support services in particular, is readily accessible to people accessing mental health services and their support people.
- Ensure all relevant policies, standards, guidelines and legislation is accessible by all staff members of the service.
- Promote a culture that supports and understands the importance of sexual safety through leadership, training and endorsement, and models mutual respect and positive relationships.
- Support staff to appropriately report and record any sexual safety incident, taking account of the incident type, whether the alleged perpetrator is a person accessing the mental health service, visitor, or staff member, and the age of the person who has disclosed the incident.

2.2. Minimum Requirements

EMHS Mental health services must:

- Develop sexual safety standards that define appropriate behaviour for the service in consultation with all members of the service.
- Provide clear information and advice to people accessing the service that takes into account their cultural background, gender, age, sexual orientation and personal experiences regarding:
 - Their rights and responsibilities in relation to sexual safety
 - The sexual safety standards that exist in the service setting
 - The process for addressing a sexual safety incident
 - How to report an incident in the event of a sexual safety incident, including sexual assault or harassment
- Ensure all frontline staff working in mental health services (including peer support workers, volunteers, security officers and other staff members) undertake training to enable them to minimise/work to prevent and respond effectively to sexual safety incidents.
- Assess the vulnerability of each person on their admission to the service, which should include any history of sexual safety incidents or incidences of sexual disinhibition.
- Ensure the Treatment, Support and Discharge Plan (TSDP) is updated and documented sensitively, including recognition of whether they are from a marginalised or vulnerable cohort i.e., Aboriginal people, ethnoculturally and linguistically diverse people, people with a disability, and people who identify as LGBTIQ+.
- Collaborate with every person accessing the service, and their support people (where

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appropriate), to develop a sexual safety plan including person-centred sexual safety risks and relevant risk mitigation strategies, and document this clearly in the TSDP.

- Respond, report and record any sexual safety incident ensuring the relevant processes are followed.
- Provide staff with an opportunity to de-brief as required when a person discloses a sexual safety incident, or they witness a sexual safety incident.

2. Assessment and management of sexual safety risk

3.1. Assessment of sexual safety risk

Some people with certain personal characteristics have increased vulnerability to sexual safety risk, including:

- Females
- People from an ethnoculturally and linguistically diverse (CALD) background
- Older people
- Young people
- Aboriginal people
- People with intellectual, developmental, cognitive and physical disabilities
- People who identify as LGBTIQ+
- People who have a history of physical and sexual abuse
- People who are heavily sedated

People who have multiple of these characteristics (known as intersectionality) may be at a greater risk of experiencing sexual safety issues.

Some people have increased risk of inappropriate sexual behaviour due to mental health conditions, an organic brain syndrome, acquired brain injury, or substance use. For example, a person with sexual disinhibition may act out their impulses.

Some people accessing mental health services may be more predatory in intent and engage in opportunistic behaviour by targeting other people who may be considered vulnerable. Deliberate tactics that may be used to select and breach the sexual safety of vulnerable people include:

- Recruiting people by means of bribes (money, cigarettes, food, and other items) or offering perceptions of care, love and intimacy.
- Utilising power/positions of power to gain confidence and trust for the purposes of gaining sexual favours.
- Maintaining accessibility by using threats to discourage the individual from disclosing the assault.
- Grooming staff by ingratiating themselves as helpful and suggesting to staff that the individual they assaulted is lying.

Assessment of sexual safety risk must be undertaken for all people who access EMHS mental health services.

Assessment of a person's sexual offending risk and sexual vulnerability should include

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present or past history of exposure to sexual abuse and/or inappropriate sexual activity as part of a developmental history including:

- Sexual history, sexual development, sexuality and/or gender orientation, Human Immunodeficiency Virus (HIV) status and any other sexually transmitted diseases.
- History of and vulnerability to sexual abuse, assault or harassment.
- History of and potential for sexual activity due to illness, emotional state or behaviour.
- History of and potential for sexually inappropriate behaviour including sexual disinhibition, sexual harassment or sexual assault.
- Any concerns relating to visitors that may contribute to the person's sexual vulnerability or risk of sexually inappropriate behaviour.
- History of and potential for violent behaviour, threats or intimidation.
- Capacity and consent related issues.
- Identification of higher risk aspects such as from a culturally and linguistically diverse (CaLD) background, identifying as LGBTIQ+, belonging to older or younger age groups, being of Aboriginal descent, experiencing mental health symptoms or disability.
- Relational/social or environmental/context risk factors, such as staff levels, mix of gender and acuity, and family situation including family and domestic violence.

3.2. Management of sexual safety risk

Where an assessment has identified that a person has the potential to engage in sexual behaviour which is inappropriate to the context or setting, the following actions should be undertaken:

- A Risk Assessment Management Plan (RAMP) and a Treatment, Support and Discharge Plan (TSDP), including sexual safety planning, should be developed in collaboration with the person (where possible) and their support people (with consent), detailing the person's risks and management strategies in relation to sexual safety.
- People noted to be at increased risk of sexual vulnerability or risk of offending are to be highlighted at inpatient review meetings or multidisciplinary team (MDT) review meetings and at handover. Staff are to consider levels of visual observation, entering a WebPSOLIS and WebPAS alert where appropriate, iSoBAR information handover and include an alert on the person's journey board where possible (due to visibility of journey boards). Include plans to manage sexual vulnerability, inappropriate sexual behaviour and consider room allocation and gender of allocated clinicians.
- All mental health services should identify sexual safety practices appropriate for their particular setting, in collaboration with all members of the service including staff, people accessing the service and their support people, clinicians and advocates.
- People identified as at risk are to be closely monitored and regularly assessed throughout their admission and at times of transition, such as movement to high or lower dependency areas, or imminent discharge.
- Consider management of co-occurring drug and alcohol use.
- Consider the impact of routine medications that may cause sedation.
- Encourage people to be involved in unit activities and programs to minimise isolation and vulnerability.
- Monitor visitor interactions for inappropriate behaviour, particularly of a sexual nature.

- Choose a therapy room with an observation window, a room close to reception and/or keep the door ajar during sessions.
- Familiarise people with their rights, responsibilities, ward rules and acceptable use of technology, such as recording and sharing of images of themselves and others online and on social media sites.

Examples of sexual safety risk management strategies for vulnerable groups

It is important to note that these examples may not apply to everybody. They may also apply to multiple groups. It is the responsibility of staff to work with people receiving care and their support people to determine what strategies best suit the individual according to their unique characteristics and circumstances.

Table 1. Examples of sexual safety risk mitigation and management strategies

Source: Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia 2020; Chief Psychiatrist's Standard for the Sexual Safety of Consumers of Mental Health Services 2024.

Group	Example strategy
LGBTIQ+	• Ask the person how they identify in relation to their gender and use the name and pronouns they prefer, including in all written and verbal communication. • Use gender neutral language when inquiring about people's relationships, for example asking if someone has a 'partner' as a default term, instead of 'boyfriend/girlfriend'.
Women	• Promote dignity by allocating a female staff chaperone during examinations and assistance with activities of daily living such as bathing and showering for females. • Offer choices about the type of support they receive, and who provides it to them (e.g. a choice between a male or female doctor, care coordinator and advocate).
Aboriginal people	• Provide culturally appropriate and sensitive material that supports sexual safety advice to people accessing mental health services and their support people. • Interpreters may be required for Aboriginal people whose first language is not English. • Ask the person whether they would like to involve cultural consultants, Aboriginal Health Liaison Officers or significant community members such as Elders in conversations about their care. • Ask whether there are any cultural protocols that need to be followed when working with them. This could mean that only people of the same gender are able to talk about certain subjects, such as discussing sexual abuse (known as 'women's business' and 'men's business').

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Culturally and linguistically diverse (CALD) people	• Engage a professional interpreter if required, but consider using one outside of the person's local community (such as a phone interpreter from interstate) to avoid feelings of shame when discussing sexual assault. • Family and friends of the person are inappropriate substitutes for professional interpreters as their interests may be different or in conflict with the person's. • Consider the literacy level of the person and be prepared to explain written information in a clear and concise way, avoiding use of medical jargon. • Develop targeted prevention strategies and responses to breaches of sexual safety that recognise and respond to the specific needs of the person relating to their cultural and religious background and beliefs.
Children and young people under the age of 18 years	• Consider their developmental stage, decision-making capacity and communication style when developing strategies to support them in keeping themselves safe. • Information and education about sexual health and sexual safety should be tailored to their communication and developmental needs.
People with intellectual, developmental, cognitive and physical disabilities	• Assess the person's communication skills and current level of knowledge and understanding regarding sexual and personal relationships to ensure they have the capacity to consent and to assess their vulnerability to sexual safety breaches. • Provide clear advice tailored to their communication and comprehension level about their rights in relation to sexual safety, particularly their ability to say 'no' to sexual activity. • If another person is required to assist with support or communication, ask the person who they would prefer to assist them.
Older people	• Ensure there are sufficient staff of appropriate gender to meet intimate personal care needs in a way that the person feels safe and treated with dignity.

3. Access to Information

Upon admission, the person accessing the service, and their support people, will be provided with clear and accessible information about their rights and responsibilities, expectations, acceptable and unacceptable behaviours, and how to seek more support, report a breach, or make a complaint.

People identified as being at risk of sexual vulnerability or sexually inappropriate behaviour will be supported to access information to enable them to effectively recognise and respond to

behaviours, both their own and other people's, that may compromise or breach their own or another person's sexual safety. The delivery of information should suit the person's needs, including options such as:

- Verbal, visual, or written information
- Providing information with support from an interpreter
- Providing information with support from cultural consultants

4. Safe environments

EMHS mental health services will provide a sexually safe environment which is:

- Trauma informed, based on the principles of safety, trustworthiness, choice, collaboration empowerment and emphasises recovery.
- Fair and equitable, available to a diverse range of individuals and free from discrimination.
- Compassionate, sensitive and promotes a culture that encourages reporting of sexual safety incidents relating to people accessing mental health services, support people and staff.
- Safe and non-judgemental for LGBTIQI+ people through actively promoting the service's acceptance of diverse sexual orientations and gender identities.

EMHS mental health services will balance personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all people receiving care, visitors and staff.

On admission/activation, people accessing the service will be provided with an orientation to the inpatient/community setting, including their right to a safe and therapeutic environment, and explanation of mutual obligations. This includes signing admission agreements outlining that sexual behaviour and activity within the service is inappropriate and not permitted, and that intimate behaviour or sexualised contact with other people accessing the service, visitors or staff is not acceptable. This includes not permitting people in each other's rooms on inpatient units.

Unless clinically contraindicated, all people accessing mental health services will be asked about their staff gender preference, and staff allocation will consider their gender preference alongside clinical need and staff availability.

Environmental designs/fit outs for mental health inpatient areas will include consideration of the Office of the Chief Psychiatrist (OCP) Sexual Safety Guidelines: e.g., bedroom doors – lockable from the inside & bedroom doors having alarms, sensory motion detectors installed in bedrooms and signage within units.

5. Response to breach of sexual safety

Disclosures about alleged sexual safety incidents, regardless of the time frame of the alleged incident, are to be addressed promptly by staff. Staff must immediately escalate all reports of alleged sexual safety incidents to the immediate line manager and the consultant psychiatrist.

Where a breach of sexual safety is identified, staff will:

- Listen and acknowledge disclosure non-judgementally.

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- Assess the person's immediate safety and wellbeing, need for medical treatment, and proactively identify the potential for re-traumatising people with trauma histories.
- Ensure the person's right to privacy and confidentiality is respected.
- If consent is provided and it is appropriate, inform the person's support people.
- Provide appropriate support/treatment and or referral to services e.g. forensic examination, Sexual Assault Resource Centre (SARC), Police.
- Review and update the person's RAMP, TSDP and sexual safety plan.
- Consider capacity and consent related issues and guidance if capacity is lacking/impaired.
- Identify and document plan for ongoing care, such as:
 - Need for specials, frequency of observations, need for medications, follow-up investigation results;
 - Consider a review of levels of visual observation;
 - Discharge planning and handover, including documentation of any sexual safety breaches and safety planning for future admissions;
 - Follow-up, including post discharge:
 - Who – e.g., GP, community mental health services
 - What – e.g., Investigation results, pregnancy testing, repeat testing for STI and BBV.
- Place an alert on PSOLIS and/or WebPAS for people with known/reported offences and/or vulnerabilities.
- Identify a lead to investigate the incident.
- Offer a debrief to ensure support for the person, at a time and place they feel comfortable. They may decline to have a debrief or may request a certain staff member they feel most comfortable with to debrief.
- Psychological support is to be offered to all staff following a reported incident, with the opportunity to debrief and/or access to the Employee Assistance Program.

All EMHS sites must have Standard Operating Procedures (SOPs) with clear and practical guidance on escalation of care in response to sexual safety concerns, including during overnight shifts or other low-staff periods.

6. Notifications

The person will be asked whether they wish to involve the Police and are to be supported if they wish to do so. They may change their mind and report to Police at any stage.

Reporting body	Who reports	What to report	When to report
Department of Communities via Mandatory Reporting Online Form (for under 18)	Mandatory reporters under the Children and Community Services Act 2004 (section 124B), including: • Doctor • Nurse • Psychologist	Suspected child sexual abuse. Refer to Mandatory Reporting Web System (MRWeb) for details. <i>Not applicable for people 18+.</i>	As soon as practicable after forming the belief that a child has been the subject of sexual abuse or is the subject of ongoing sexual abuse.

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Reporting body	Who reports	What to report	When to report
The Chief Psychiatrist	Nominated relevant mental health staff	The Chief Psychiatrist is to be notified of any incidents as per the Office of the Chief Psychiatrist Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist – Policy for Public Mental Health Services , including retracted disclosures, alleged sexual safety incidents, or sexual assault.	As soon as practicable, ideally within 48 hours of the event.
Datix CIMS (as applicable)	Nominated relevant mental health staff	Consideration should be made whether to report the breach as a clinical incident through the DATIX Clinical Information Management (CIM) system and Combined Hazard or Incident Report (CHoIR). All incident indicators available on the incident management system must be completed to ensure consistent reporting.	As soon as practicable.
Sex Assault Squad (WAPOL) – if the person chooses to report to police	The person who was subject to the sexual assault, supported by their support people and mental health staff. Refer to Reporting sexual offences information for People who are 16 years and older (WA Police information booklet).	Contact the Sex Assault Squad (WA Police) on 08 9428 1600 to report sexual assault. If required, staff should seek advice from the relevant co-director or on call executive (after hours) regarding reporting the incident to the Police in the public interest and/ or the need for forensic examination.	As soon as practicable after the person decides to make the report, noting that there are no timeframes dictating when a report must be made to police after the event occurred.
WorkSafe WA Refer to WorkSafe WA Incident Notification - Interpretive Guideline	Nominated relevant mental health staff, via Work Health and Safety	A 'serious injury or illness'. Defined as: - Requiring immediate treatment as an in-patient in a hospital, for any duration, even if the stay is not overnight or longer - Injury or illness that a medical practitioner considers likely to prevent the person from being able to do their normal work for at least 10 days	Immediately after becoming aware it has happened

7. Documentation

All sexual safety incidents must be clearly and accurately recorded in the medical record of the person who has disclosed the incident, without identifying the alleged perpetrator; and in the medical record of the person who breached someone else's sexual safety without identifying the person reporting the incident.

The incident must also be documented on a separate record, stored separately from the person's medical record, which includes full details of the incident. This record should document what is reported, including the identity of the alleged perpetrator as stated by the victim and the details of any witnesses. It should also contain the investigation and outcomes following the service's policies for serious clinical incidents and complaints.

An alert is to be placed on PSOLIS/WebPSOLIS, and a risk flag is to be placed on ICM and WebPAS for all inpatients, within 24 hours of the incident being reported.

8. Training and Education

Mental health services need to foster a culture that supports people to report sexual safety incidents in a trustworthy, safe and transparent manner. Training and education will play a critical role in supporting services to respond positively and respectfully to disclosures and provide reassurance that any sexual safety incident/breach is considered important and will be investigated.

9.1. EMHS Staff

Training is essential for all staff working in mental health services (including peer support workers, volunteers, security officers and other staff members) around sexual safety, disclosures/reports of sexual safety and incidents/breaches. Staff awareness of the impact can inform clinical practice and improve treatment outcomes for people accessing mental health services:

- EMHS mental health staff must be familiar with this policy and site processes and procedures that outline their responsibilities in preventing and responding appropriately to sexual safety breaches/incidents disclosures/reports.
- EMHS mental health staff are to be provided with appropriate training on gender sensitive practice and managing sexual safety within mental health settings on orientation to the work area, including education on how to discuss sexual safety concerns with people accessing mental health services, carers and guardians, and support people in a sensitive and trauma-informed manner.
- EMHS mental health staff must complete any mandatory training related to sexual safety and gender sensitive practice, including refresher training at the required intervals, according to their site's training requirements.
- EMHS Training that supports sexual safety and trauma-informed care includes:
 - Maintaining Inpatient Sexual Safety
 - Responding to Disclosures of Sexual Assault (SARC)
 - Mandatory Reporting of Child Sexual Abuse

9.2. Feedback

Sites should implement a process for regularly seeking and incorporating feedback regarding sexual safety into service planning and continuous improvement efforts, including feedback obtained from post-incident debriefing.

9. Compliance monitoring and legislative penalties

10.1. Compliance monitoring

The EMHS Executive Director Mental Health is responsible for ensuring EMHS mental health staff are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

- Site based sexual safety trends are to be tabled at every EMHS Mental Health Leadership Committee meeting for monitoring.
- Sexual Safety Clinical Incident Reports, with details on incident responses and lessons learned from incidents over the previous six-month period, are to be tabled every six months.

10.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

10. Legislation

- [Mental Health Act \(MHA\) 2014](#)
- [Criminal Code Act 1913](#)
- [Criminal Investigations Act 2006](#)
- [Guardianship and Administration Act 1990](#)
- [Health Practitioner Regulation National Law \(WA\) Act 2010](#)
- [Children and Community Services Act 2004](#)
- [Commonwealth Aged Care Act 1997](#)
- [Work Health and Safety Act 2020](#)
- [Children and Community Services Act 2004](#)

11. Additional references

12.1. WA Health policies

- MP0122/19 [Clinical Incident Management Policy](#)
- MP 0127/20 [Discipline Policy](#)
- MP 0125/19 [Notifiable and Reportable Conduct Policy](#)
- MP 0130/20 [Complaints Management Policy](#)
- MP 0155/21 [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services Policy](#)

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- MP 0124/19 [Code of Conduct Policy](#)
- [Procedure for Responding to a Recent Sexual Assault](#)
- [Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy](#)
- [Guidelines for Protecting Children](#)
- MP 0175/22 [Consent to Treatment Policy](#)
- MP 0181/24 [Safety Planning for Mental Health Consumers Policy](#)

12.2. EMHS policies

- [Chaperone Policy](#)
- [Notification requirements under the WA Mental Health Act 2014 Policy](#)
- [Volunteer Policy](#)
- [Mental Health Advocates Policy](#)
- [Critical Incident Stress Debriefing Policy](#)
- [Acute Response Mental Health Emergencies/Crisis Policy](#)

12.3. Other

- [WA Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia 2020](#)
- [Chief Psychiatrist's Standard for the Sexual Safety of Consumers of Mental Health Services 2024](#)
- [Chief Psychiatrist's Sexual Safety Guidelines Poster](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [King Edward Memorial Hospital- Sexual Assault Resource Centre \(SARC\)](#)
- [SMHS FSFHG Sexual safety in Mental Health Policy 2024](#)
- [NMHS Sexual safety in Inpatient Services Policy 2023](#)